

TACTICAL FIELD CARE

PRIMARY SURVEY (MARCHH)

MASSIVE HAEMORRHAGE (M)

ALL EXTERNAL MASS HAEM CONTROLLED & PELVIC BINDER APPLIED IN BLUNT POLYTRAUMA
SEE TRAUMATIC HAEMORRHAGE MANAGEMENT (Page 8)

AIRWAY (A)

SEE ADVANCED AIRWAY MANAGEMENT (Page 10), MAXILLOFACIAL HAEMORRHAGE (Page 12)
LOW THRESHOLD FOR AIRWAY BURNS (Page 16)

RESPIRATION (R)

SEE TRAUMATIC CHEST INJURIES (Page 14), ESHAROTOMY FOR TORSO (Page 26)
FOLLOW JRCALC FOR ASTHMA GUIDELINES

CIRCULATION (C)

TRAUMATIC HAEMORRHAGE MANAGEMENT (Page 8), CRUSH INJURIES (Page 20)
BURNS MANAGEMENT (Page 16)

HEAD (H)

HEAD INJURIES (Page 18), ENVENOMATION (Page 44)
POISONING OR TOXIC INJURIES (Page 46)

HEAT (H)

HEAT ILLNESS (Page 40)
HYPOTHERMIA (Page 42)

PACKAGING FOR TRANSFER/MEDEVAC

CONCOMITANT PACKAGING ALONG WITH PRIMARY SURVEY (ASSISTANCE REQ'D)

EARLY AND APPROPRIATE ANALGESIA (ANALGESIA LADDER)

PLACE PACKAGING KIT UNDER PATIENT WHEN LOG ROLLING TO CHECK BACK

MAINTAIN NORMOTHERMIA - NO COLD TRAUMA PATIENTS

MINIMISE MOVEMENT FOR CLOT AND SPINE PROTECTION (EARLY VACUUM MATTRESS OR
SCOOP STRETCHER IF AVAILABLE)

SPLINT OR REDUCE FRACTURES IF PRACTICABLE

ENSURE ALL LINES ARE SECURE AND CONTINUOUS MONITORING IS ATTACHED